



Zuclopenthixol Acetate (Clopixol Acuphase®) Intramuscular Injection SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	All inpatient areas	Multidisciplinary staff

Purpose

This standard operational procedure (SOP) aims to ensure the safe prescription, administration and monitoring of patients, post zuclopenthixol acetate (Clopixol Acuphase®) intramuscular injection (IMI) within inpatient areas in mental health (MH) and general health.

Links within document

- [Appendix I: Post Acuphase® \(Zuclopenthixol Acetate Intramuscular Injection\) Administration Vital Observations Flowchart](#)

Links to relevant documents

- Clopixol® product information: MIMS online. [Full Product Information](#)¹

Definitions

Zuclopenthixol acetate (Clopixol Acuphase®)	An intermediate acting antipsychotic injection indicated for acute psychosis and acute mania and/or with extreme aggression (e.g. require repeated IMI PRN medication for agitation and arousal) where other treatment modalities have been ineffective.
Corrected QT interval (QT_c)	The length of the QT _c adjusts the QT interval (the time between the start of the Q wave and the end of the T wave on the electrocardiogram [ECG]) correctly for heart rate extremes. ²
QT prolongation	Medications (e.g. Clopixol Acuphase®) can cause prolongation of the QT interval and have been associated with life-threatening arrhythmia. ² The risks and benefits of treatment are to be carefully considered when prescribing concomitant medications (e.g. other antipsychotics, methadone, and macrolide antibiotics) that can also have this effect. ²

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Version: 3.2

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Presentation

- 50 mg/1 mL or 100 mg/2 mL glass ampoules (clear, yellow viscous oil)

Do not confuse with zuclopenthixol decanoate (Clopixol Depot®) a long acting IM injection.

Indications

- Acute psychosis, including mania, and exacerbations of chronic psychosis, particularly when the patient is hostile or aggressive
- Not to be used for rapid tranquilisation or acute sedation due to delay in sedative action of at least 2 hours.

Reserve for those patients who:

- have required repeated IM PRN medication for agitation and arousal
- have had sufficient time for assessment of response and/or side effects to previously injected medications (minimum 60 minutes for IM and 15 minutes for intravenous [IV]), or
- have previously received Clopixol Acuphase® and have shown good tolerability and response to it.

Common side effects

- Sedation, extrapyramidal side effects (EPSE), hypotension and tachycardia are commonly encountered adverse effects
 - Sedation can be expected to occur in the initial 2 to 3 hours post dosing and be maximal at 12 to 36 hours, but this pattern may vary
- Severe adverse reactions requiring immediate medical attention may occur and are difficult to predict. Therefore, the evaluation of a patient's tolerance and response, and establishment of adequate maintenance treatment requires careful stabilisation of each patient under continuous, close medical observation and supervision, especially in older persons¹
- Significant effects can last for up to 72 hours, though levels can still be detected in the serum after this time. Care should be exercised if additional doses of antipsychotics/other medications with central nervous system or respiratory depressant effects (e.g. benzodiazepines, opioids) are prescribed.

Contraindications

Clopixol Acuphase® contraindications include:

- known hypersensitivity to Clopixol Acuphase® or any of the excipients (coconut oil)
- acute alcohol, barbiturate or opiate intoxication
- circulatory collapse
- depressed level of consciousness due to any cause, coma, suspected or established subcortical brain damage
- blood disease (dyscrasias)
- phaeochromocytoma (a rare tumor of adrenal gland tissue)
- leukopenia and/or previous agranulocytosis^{1, 2}

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Common side effects – Contradictions cont'd.

Clopixol Acuphase® is preferably **NOT** to be used:

- in an attempt to hasten the antipsychotic effect of other antipsychotics
- at the same time as other parenteral antipsychotics or sedation which is difficult to reverse
- as a test dose for zuclopenthixol decanoate depot
- or in patients who:
 - can accept oral medication
 - are pregnant
 - have Parkinson's disease or dementia with Lewy bodies
 - have severe hepatic or renal impairment
 - have infection of unknown origin.

Precautions for the use of Clopixol Acuphase® include:

- dyskinesia - the possibility of the development of irreversible dyskinesia for patients on prolonged treatment
- photosensitivity reactions
- anaphylactoid reactions
- convulsive disorders/seizures
- mild renal/liver Impairment¹
- organic brain syndrome.²

Avoid verbal, faxed requests or any orders written in advance.

Procedure details

Royal Perth Hospital (RPH) wards to access zuclopenthixol acetate (Clopixol Acuphase®) and Post Acuphase® Monitoring Checklist from Ward 2K). ED and general health wards can access supply via Pharmacy.

Exception: Mental Health Unit has own IMPREST stock. The Bidi Wungen Kaat Centre (Transitional Care Unit [TCU]) - refer to the agreement that guides medication dispensing/supply.

Dosage and administration

Note: Clopixol Acuphase® treatment is not for rapid tranquilisation and use is a consultant-led decision. Trainees/registrars or non-Mental Health teams are to discuss Clopixol Acuphase® with a Consultant Psychiatrist/Duty Psychiatrist. The outcome of this discussion and Consultants name should be documented in the patient's healthcare record . Consultant Psychiatrist/Duty Psychiatrist approval is required for each dose (typically once only).

Pre-administration checks

Prior to **every administration** of Clopixol Acuphase® the medical officer (MO) is to clinically assess the patient in consultation with the Psychiatry Consultant to determine the risk versus benefit of this therapy.

- In MH = Duty Medical Officer (DMO)
- In General Health = CL Psychiatry SAFE/^{Be} SAFE

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Procedure details – Pre-administration checks cont'd.

Assessment is to include:

- patient's baseline temperature (T), heart rate (HR), blood pressure (BP), respiration rate (RR), oxygen saturation (SpO₂) and recent vital sign trends (where available)
- baseline ECG performed on admission/at any stage during the admission
- an additional ECG would be indicated in susceptible patients (with hypokalaemia, hypomagnesaemia or genetic predisposition), with a history of cardiovascular disorders, evidence of toxicity/side effects, or if maximum doses are approached
- every effort is to be made to obtain baseline and ongoing assessments including escalation to SAFE/^{Be}SAFE where possible when patients are refusing
- if there are no vital signs, ECG, or physical examination available (baseline or ongoing at any stage of the admission), when considering a decision to use Clopixol Acuphase[®] the Psychiatry Consultant must evaluate the risks and benefits of Clopixol Acuphase[®] and clearly document this analysis and final decision in the patient medical record
- A/hrs DMO/SAFE/^{Be}SAFE to summarise advice from oncall consultant (include name) in patient integrated notes
- ensure only 1 dose of Clopixol Acuphase[®] **50 mg (1 mL) to 150 mg (3 mL)** is prescribed at a time (in 24 hours). Record on the Once Only section of the *WA Hospital Medication Chart (HMC)*
- do not prescribe on an "as required" (PRN) basis
- the prescribing clinician is to provide patients written information on Clopixol Acuphase[®] at commencement of treatment (provide [WA Department of Health CHOICE Medication leaflet](#)) and relevant information at every clinical review and document this discussion in the patient's healthcare record
- (see **Medication Review Policy**).

[illegible]

Figure 1: WA HMC

- Note that the first injection is usually the most sedating and has an antipsychotic action lasting for 2 to 3 days after administration by **deep IMI** into a large muscle (i.e. gluteal muscle or lateral thigh)

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Procedure details – Pre-administration checks cont'd.

- Repeat doses may be prescribed preferably every 2 to 3 days, if necessary (a repeat dose may be needed 24 hours after the first dose in some cases)
 - The maximum dose for older adults is 100 mg (2 mL), depending on the patient's severity of illness
 - Females and older adults may require lower doses of 25–50 mg ^{2,3,4}
- Ensure the maximum accumulated dosage for **consecutive doses over a 2 week period does not exceed 400 mg** and the total number of injections does not exceed 4. It is expected that 1 or 2 injections will be sufficient
- Treatment should be for a maximum of 2 weeks before switching to an alternative zuclophenthixol formulation or another antipsychotic
- Commence oral Clopixol® tablets 2 to 3 days after the last injection of Clopixol Acuphase®, if required. Zuclophenthixol decanoate can be given with the last injection of Clopixol Acuphase®. **Note:** Clopixol Acuphase® can be mixed with zuclophenthixol decanoate in the same syringe ^{1,2,3}
- Consider the prescription of PRN anticholinergic agents (e.g. benztropine) in case of EPSE
- Withhold all other parenteral antipsychotics for 24 hours when patients are receiving Clopixol Acuphase® including medications with a PRN dose frequency
- Consider that co-administering Clopixol Acuphase® with a benzodiazepine or antipsychotic short acting IM injections should not be routine practice. Effects of any previously injected medication should be assessed before administering Clopixol Acuphase® (minimum of 60 minutes post IMI and 15 minutes for intravenous [IV]). In the event that co-administration of a short acting IMI and Clopixol Acuphase® is deemed necessary, consideration should be given to the risk of excessive sedation or other adverse effects. The rationale for co-administration and any increased monitoring requirements must be documented.

Note: Do not confuse with Zuclophenthixol Decanoate (Clopixol Depot® a long-acting IM injection).

Post injection requirements

Clinicians are to:

- Ensure close monitoring of patients, post injection
 - Document vital signs on the *Adult Observation and Response Chart (AORC)* including physiological observations and level of consciousness
 - Vital signs are most importantly monitored at the 24 and 36 hour mark when serum levels of Clopixol Acuphase® peak
 - Record when observations and ECG are completed on the Post Acuphase® (Zuclophenthixol Acetate Intramuscular Injection) *Monitoring Checklist Form*
 - Place the checklist in the patient's healthcare record
 - (Bentley Health Service [BHS]) or bedside notes [general health])/ RPH

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Procedure details – Post injection requirements cont'd.

Mental Health Areas	General Health Areas
Review: <i>Risk Assessment Management Plan (RAMP)</i> on WebPSOLIS <i>Progressive Risk Assessment (PRA)</i> and the Agitation and arousal rating scale as per the <i>WA Agitation and Arousal PRN Medication Chart</i>	Review the current <i>Behavioural Observation Form (BOF)</i>
Review ECG for QTc interval prolongation at 24 hours and 48 hours post injection	Review ECG for QTc interval prolongation at 24 hours and 48 hours post injection
Add Medication alert to PSOLIS/webPSOLIS alert	
<i>Complete Blood Count/Continuation Form</i>	<i>Complete Blood Count/Continuation Form</i>
Document completion of Mental State Examination (MSE) in patient's healthcare record	Request CL Psychiatry to re-assess MSE and document in patient integrated notes

- Document responses to precautions and contraindications in the patient's healthcare record
- Document administration of Clopixol Acuphase® on the *WA HMC* and in the integrated notes and for MH patients, in the Psychiatric Services Online Information System (PSOLIS)/WebPSOLIS Client Management Plan
- Document in the *Comprehensive Care Plan (CCP)* under the 'individualised strategies' section and initial in the column after administration
- Record the need for Clopixol Acuphase® in *Notifications and Clinical Summary (NaCS)*
- Ensure patient is flagged on iClinical Manager (iCM) as per criteria in the **Behaviours of Concern (Preventing, Recognising and Responding) Clinical Practice Standard (CPS)** and **Clinical Handover Policy**. This facilitates SAFE/BeSAFE review

Nursing staff are to:

- Request MO/DMO/Consultant to review the patient's EPSE before additional antipsychotics, benzodiazepines and/or sedating medications are able to be given within this specified period
- MO/DMO/ Consultant (MH) / CL Psychiatry (General Health)/ On Call Consultant (after hours) are to document review in the patient's healthcare record
- Contact the patient's MO/DMO/consultant in the instance where there is no documentation evident.

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Procedure details cont'd.

Recommended frequency times for vital observations if clinically possible

Frequency times for vital signs monitoring
After the first injection, every 15 minutes for the first hour, then hourly for the next 3 hours
2 hourly for the next 8 hours coinciding with the anticipated period of maximum sedation, then 4 hourly for 48 hours
More frequently, or longer monitoring, as clinically indicated, or at the discretion of the MO/DMO/consultant/On Call Consultant (if after hours)
Monitoring is of particular importance after the onset of sedative effects, every 4 hours up to 48 hrs post injection, with maximum sedation around 8-12 hours and at the 24hr and 36hr mark when serum levels of Clopixol Acuphase® peak
- Monitoring should include observation for excessive sedation, snoring, pyrexia, muscle rigidity or other EPSE, particularly at 12 hours post Clopixol Acuphase®, when sedation is at its most significant, or at 24 - 36 hours, post dose (corresponding with peak serum levels) - Consider repeat ECG if observations are outside of expected parameters or patient appears excessively sedated - Early medical/SAFE/BeSAFE review of obese patients, those with known or suspected obstructive sleep apnea should be considered
As a guide to monitoring, refer to the <i>Post Acuphase® (Zuclopenthixol Acetate Intramuscular Injection) Monitoring Checklist Form</i>
*If vital signs indicate any deviation in patient parameters, or if the nurse is concerned about the patient's condition, follow the actions/interventions as outlined in the AORC. If concern persists after following these actions /interventions, staff are to continue to escalate by activating a Code Blue (Medical Emergency Team [MET]) as per criteria. Refer to Medical Emergencies and Cardiorespiratory Arrest Management (MET) CPS and Vital Signs for Adults Nursing Practice Standard (NPS)

For MH Division patients, RPBG staff to use versions of the Statewide Standardised Clinical Documentation (SSCD) that are on PSOLIS. For documents that are not available on PSOLIS, staff must complete the SSCD documents in paper-based or sanctioned writable PDF format. Refer to the **WA Health Triage to Discharge Guidelines** for details of which SSCD documents are available on PSOLIS.

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Procedure details cont'd.

Monitoring / side effects

If patient:	Actions/interventions
Is uncooperative	Document all attempts to monitor and level of consciousness, including the patient's refusal of observations on the AORC and in the patient's healthcare record
During the night Maximum period between full observations e.g. 4 hours	Monitor the patient's respiratory function on an hourly basis, including: - respiratory rate - rise and fall (depth) of chest Note presence of any sounds suggestive of laboured, obstructed or impaired breathing, e.g. wheezing, snoring. - If there are signs of laboured breathing, request a review of the patient - In MH areas: contact the Psych MO/^{Be}SAFE/SAFE - In General Health areas: contact the Medical Registrar
Has low respiratory rate less than 10 breaths per minute/evidence of respiratory distress not likely due to sedation	Make attempts to wake the patient Initiate code blue if there is difficulty arousing the patient. Refer to the MET CPS.
Evidence of airway obstruction (e.g. snoring sounds, central cyanosis)	Make attempts to reposition airway (chin lift) or encourage patient to reposition self. If not resolving, seek medical/ ^{Be} SAFE/SAFE review Initiate code blue if there is difficulty rousing the patient. Refer to the MET CPS .

Compliance monitoring

Compliance with this document will be monitored by the relevant staff designations in accordance with standard clinical incident review requirements.

- Zero reports on Datix Clinical Incident Management System (CIMS) and/or Combined Hazard or Incident Report (CHoIR) and/or Notifiable incidents to Chief Psychiatrist (if required for MH patients) involving non-adherence to this SOP
- 100% of all prescriptions of Clopixol Acuphase® will comply with this SOP
- MH Inpatient Wards Acuphase monitoring Audit that is reported 6 monthly to the MH Executive and Lead by the RPBG Clinical Audit Team.

Facilitator

Marika Spaseska, Policy Officer, Education and Innovation, RPBG

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Authors / acknowledgements

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Richard Stewart, Director of Clinical Training, MH Division, RPBG

Vu Kha, Supervisor, BHS Clinical Pharmacy, MH Division, RPBG

Fiona MacDonald, MH Clinical Standards Nurse, Safety & Quality Department, MH Division, RPBG; 2020.

Review authors

Vu Kha, Supervisor, BHS Clinical Pharmacy, Mental Health (MH) Division, RPBG

Dr Vinesh Gupta, Medical Co-Director, MH Division, RPBG

Dr Jai Nathani, Consultant Psychiatrist, MH Division, RPBG

Dr David Stevens, Psychiatrist, Inpatient Clinical Lead, MH Division, RPBG

Carmen Fuentes Vasquez, ED Consultant Psychiatrist, RPBG

Lucia Gillman, A/ Nursing Director, Education and Innovation, RPBG

Hannah Midani, Physical Health Care CNS, MH Division, RPBG

Lyn Grattidge, Clinical Nurse Specialist, Infection Prevention & Management, RPBG

Julia Hook, Coordinator of Pharmacy, BHS Pharmacy, RPBG

Lucy Turner, A/Clinical Governance Nurse, MH Division, RPBG

Shannon Wilson, Nurse Unit Manager (NUM), Ward 7, BHS, MHD, RPBG

Scott Beattie, Senior Pharmacist, SMART Infusion Pump Systems and Drug Error Reduction Software, RPH Pharmacy, RPBG

Feedback received

Judith Cuff, A/Safety and Quality Coordinator, RPBG

Fiona Macdonald, A/Nurse Educator - Clinical audit, Education and Innovation, RPBG

Cheryl Burnaby, A/MET Coordinator, BHS, RPBG

Neil Rigby, NUM, Mental Health Unit, RPH, RPBG

Thomas Hartnett, Pharmacist, Department of Pharmacy, Royal Perth Hospital Infection Prevention and Management Unit

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Related policies, practice standards, clinical guidelines

RPMG Policy Hub

- [Medical Review Policy](#)
- [High Risk Medication Policy](#)
- [Medication Preparation and Routes of Administration CPS](#)
- [Medical Emergency and Cardiorespiratory Arrest Management CPS](#)
- [Vital Signs for Adults Nursing Practice Standard](#)
- [Comprehensive Care Policy](#)
- [Admission, Discharge and Transfer SOP](#)
- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Special and Supportive Observations Policy](#)
- [Behaviours of Concern \(Prevent, Recognise and Respond\) CPS](#)
- [Recognising and Responding to Acute Deterioration Policy](#)
- [Recognising and Responding to Acute Deterioration in Mental State Cognition and Behaviour Policy](#)
- [Clinical Handover Policy](#)
- [Prescribing Policy](#)
- [Bidi Wungen Kaat Centre \(Transitional Care Unit \[TCU\]\) Referral, Admission, Discharge and Transfer Policy](#)

EMHS Policy Hub

- [Acute Response Mental Health Emergencies/Crisis Policy](#)
- [Inter-Hospital Patient Transfer Policy](#)
- [Transportation of Mental Health Consumers Policy](#)
- [Psychiatric Service Online Information Service \(PSOLIS\): Communication of Risk \(Alert\) Guideline](#)

WA Health Policy Hub

- [High Risk Medication Policy](#)
- [Medication Review Policy](#)
- [Medicines Handling Policy](#)
- [Consent to Treatment Policy](#)
- [Recognising and Responding to Acute Deterioration Policy](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)
- [Clinical Incident Management \(CIM\) Policy](#)
- [Statewide Standardised Clinical Documentation \(SSCD\) Policy](#)
- [‘Triage to Discharge’ Mental Health Framework for SSCD](#)

Other

- [WA Department of Health CHOICE Medication leaflet](#)
- [WA Mental Health Act \(2014\)](#)
- [Chief Psychiatrist's Standards for Clinical Care 2015](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [Policy for Reporting of Notifiable Incidents to the Chief Psychiatrist](#)

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Related forms and stickers

- *WA Hospital Medication Chart (HMC)*
- *Adult Observation and Response Chart (AORC)*
- *Monitoring Checklist Form*
- *WA Agitation and Arousal PRN Medication Chart*
- *Post Acuphase® (Zuclopenthixol Acetate Intramuscular Injection) Monitoring Checklist Form*
- *Blood Count/Continuation Form*
- *Behavioural Observation Form (BOF)*
- *Comprehensive Care Plan (CCP)*
- *Risk Assessment Management Plan (RAMP)*
- *Progressive Risk Assessment (PRA)*
- *Notifications and Clinical Summary (NaCS)*

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021); Standard 4: Medication Safety Standard; Standard 5: Comprehensive Care Standard; Standard 6: Communicating for Safety; Standard 8: Recognising and Responding to Acute Deterioration

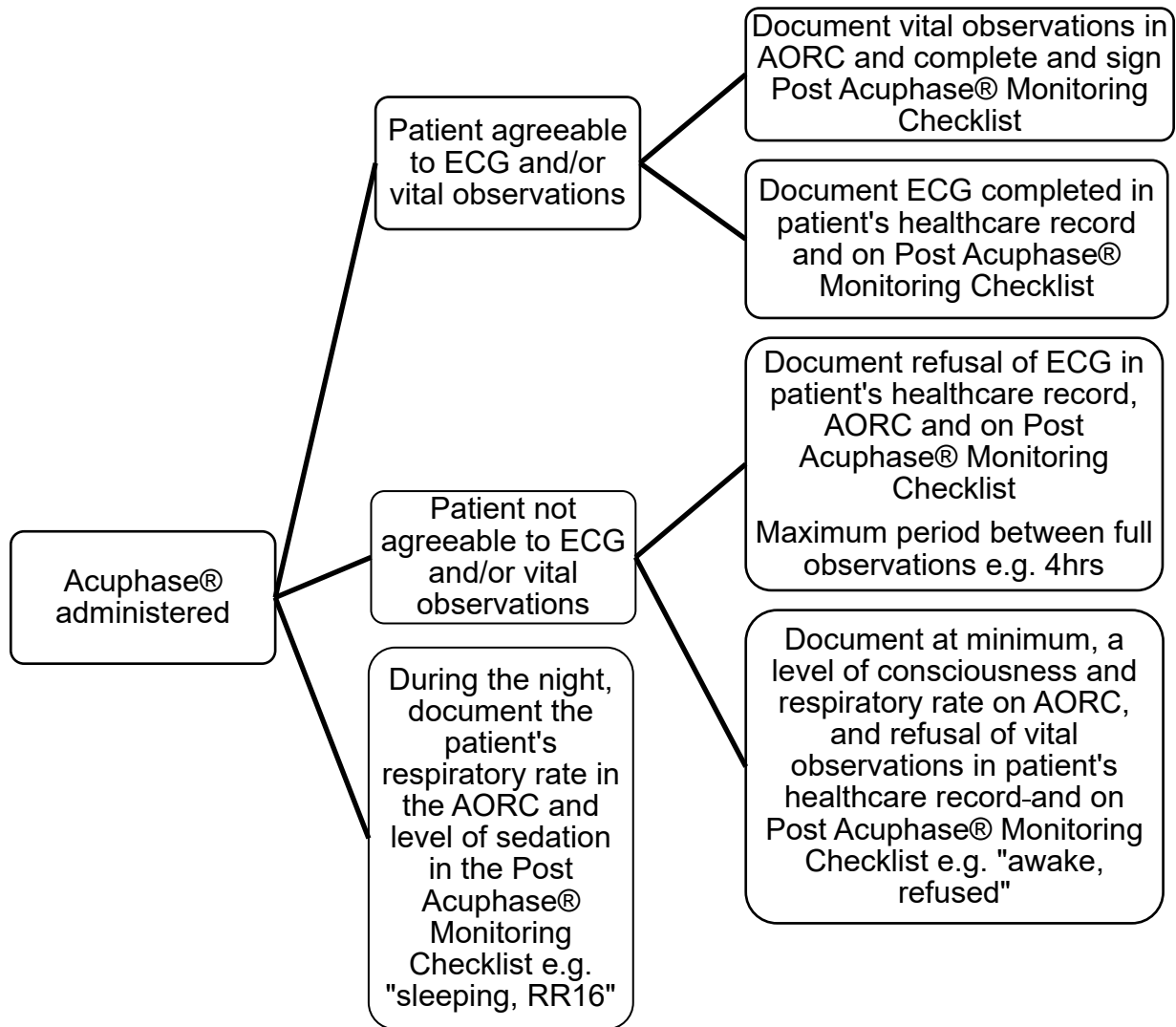
NSMHS Standards (2010): Standard 2: Safety; Standard 6: Patients; Standard 8: Governance, leadership and management; Standard 10: Delivery of care

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Appendix I: Post Acuphase® (Zuclopenthixol Acetate Intramuscular Injection) administration vital observations flowchart

Use the flowchart in conjunction with the Post Acuphase® (Zuclopenthixol Acetate Intramuscular Injection) Monitoring Checklist Form to support accurate documentation of patient's vital observations.



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